Refusal of Transfer

Patient Refuses Transport

Ensure 2x Sets of Vital Signs have been taken
(at least 5 minutes between assessments)
Including:

- Heart Rate
- Respiratory Rate
- Blood Pressure
- Blood Glucose (RBS)
- Temperature
- 12-Lead ECG (if clinically relevant)

Ensure the patient fully understands the risks of not going to hospital by ambulance

Decision Making
Capacity Assessment

Failed Capacity Assessment
and
Patient still refusing transport

Consider Tele EMS

Notify Dispatch Duty Officer

Consider Requesting Police Assistance

Passed Capacity Assessment
and
Patient still refusing transport

Patient Capable to Refuse Transport

Complete Refusal form of ePCR

- Patient signature required
- Ensure all information pertaining to the refusal is documented

Capacity Assessment

If **all** the below criteria are met, the patient has capacity to refuse transport.

- Understanding
- Appreciation
- Reasoning
- Confirmed Expression

Patient Care Goals

1. Ensure patients refusing transport to hospital have the capacity to make informed decisions while respecting their autonomy.
2. Provide comprehensive education on risks, alternatives, and follow-up care to mitigate potential adverse outcomes.
3. Document all assessments, discussions, and refusal processes thoroughly to ensure legal compliance and continuity of care.

Patient Inclusion & Exclusion Criteria

Inclusion Criteria

- Patients who decline a recommendation of transport to a medical facility after EMS evaluation and/or treatment, including:
    - Those with minor, stable conditions (e.g., resolved hypoglycemia, mild allergic reactions).
    - Those with potentially serious conditions (e.g., chest pain, altered mental status) but refusing transport against medical advice.
- Patients requiring assessment of decision-making capacity and education on risks of refusal.

Exclusion Criteria

- Patients with no EMS contact or evaluation (e.g., no medical complaint reported).
- Patients lacking capacity to refuse transport (e.g., severe intoxication, unresponsiveness, or minors without guardian consent) who require mandatory transport.
- Patients who have not been recommended transfer to hospital by ambulance (see Treated at Scene CPG).

Key Patient Assessment Points

1. **Decision-Making Capacity**
    - Assess patient's ability to understand their condition, proposed treatment/transport, and risks of refusal.
    - Evaluate for factors which may impair capacity (e.g., intoxication, altered mental status, language barriers, or psychiatric conditions).
2. **Risk Identification**
    - Identify the severity of the patient's condition and potential risks of refusing transport (e.g., deterioration, delayed diagnosis).
    - Confirm absence or presence of red flags (e.g., unstable vital signs, neurological deficits, or chest pain).
3. **Patient Understanding**
    - Verify the patient comprehends the recommendation for transport, potential consequences of refusal, and follow-up care options.
    - Assess need for additional education or clarification to ensure informed refusal.
4. **Support System and Follow-Up**
    - Determine the presence of a responsible caregiver or support system to monitor the patient post-refusal.
    - Confirm access to follow-up care (e.g., primary care, emergency contact, or pharmacy).

Patient Management & Interventions

1. **Primary Survey**
    - Ensure airway patency, adequate breathing, and circulation as per the Universal Care CPG to confirm clinical stability.
    - Address immediate life-threatening conditions before discussing refusal.
2. **Secondary Survey**
    - Conduct a focused examination as per the Universal Care CPG to identify underlying conditions and assess stability.
3. **Refusal Management Interventions**
    - **Capacity Assessment**
        - Evaluate the patient's decision-making capacity using these four steps. All of these must be met to be deemed capable:

- **Check Understanding**: Explain the situation and risks simply (e.g., "You could get worse, we suggest hospital") then ask them to repeat it back.
- **Test Appreciation:** Ask if they see their own risk (e.g., "Why do you think you are okay now?") to confirm they grasp their condition.
- **Assess Reasoning:** Ask them to weigh options (e.g., "What happens if you stay? What's your plan?") and look for logical thinking.
- **Confirm Expression:** Ask for their decision (e.g., "Do you want to go to hospital?") and ensure it's clear and consistent when re-asked after a minute.

4. **Failed Capacity Assessments**
   - If the Capacity Assessment is failed, the patient is not competent to refuse transfer and must be transferred to hospital.
   - If the patient is still refusing transfer, inform Tele-EMS.
     - Tele-EMS may instruct the administration of sedation.
   - The Dispatch Duty Officer should also be informed.
   - Police attendance at the scene may aid in persuading the patient to go to hospital.
   - Police assistance will be required if patient restraint is needed.
5. **High-Risk Refusals**
   - For serious conditions (e.g., chest pain, neurological deficits), or Red Priority patients:
     - Involve Tele EMS, and/
     - Document extensive efforts to persuade transport, and/
     - Inform the Dispatch duty officer

Patient Safety Considerations

1. Elderly Patients (>65 years)
   - Elderly patients may have atypical presentations or comorbidities increasing risk; strongly encourage transport for serious complaints.
   - Assess for social support (e.g., caregiver, home safety) to ensure safe disposition post-refusal.
2. Pediatric Patients (<13 years)
   - Require guardian consent for refusal; transport if guardian is absent.
   - Involve a parent or guardian to confirm capacity and ensure follow-up care.
3. Mental Capacity
   - If capacity is impaired (e.g., intoxication, psychiatric crisis), transport unless capacity is restored.
   - Use interpreters or translation tools for language barriers to ensure informed refusal.

Clinical Insights & Additional Guidance

- **Vital Signs Assessment**
  - Three complete sets of vital signs are required to be taken.
  - A 12-lead ECG should be performed in patients where a cardiac issue may be related to complaint or if ECG changes may identify other pathology (such as electrolyte imbalance).
- **Disposition Considerations**
  - High-risk refusals require Tele EMS consultation, extensive documentation, and patient education to mitigate liability.
- **Police or Third-Party Refusal**
  - If transport is refused by a third party (e.g., police), this must be escalated to Tele-EMS, shift in-charge, duty officer and fully documented, including efforts made by EMS staff.

Documentation

- Complete a comprehensive electronic Patient Care Record (ePCR), capturing the following elements to support continuity of care. Including:
  - Identification: Patient's full name, age, and gender.
  - Mechanism / Medical Complaint: Reason for EMS activation and complaint for which transfer is being refused.

- Injuries / Information: Pertinent history, prior assessments, and any findings indicating medical risk.
- Signs: Documented vital signs (two complete sets), AVPU or GCS, pain score if applicable, mental status.
- Treatment: All EMS-provided interventions prior to refusal (e.g., oxygen, medications, wound care).
- Allergies: Known allergies reported by the patient or found in records.
- Medications: Current medications that may influence the patient's clinical decision-making.
- Background: Relevant medical, psychiatric, or social history impacting decision capacity.
- Other: Capacity assessment outcome, education and advice provided at the refusal form, and caregiver details if applicable.

Key Performance Indicators

1. Ensure all refusal cases have a complete refusal documentation form.

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Airway
&
Breathing